

The Physician–Patient Relationship

- Encounter between physician and patient always remains a key element...and so is their relationship

Parsonian Model:

- Talcott Parsons (1951) pioneered efforts at explaining the sociocultural foundation of health care...physician-patient relationship is a sub-system
- He believed that physician–patient relationship is inevitably asymmetrical (three circumstances for which physicians play powerful role)

develop or be the first to use or apply (a new method, area of knowledge, or activity)

Inevitably means in a way that cannot be avoided or escaped



Professional prestige:

Physician's medical **expertise**, years of **training**, and the societal **legitimation** of the physician as the ultimate authority on health matters.

conforming to the law or to rules

Situational authority:

It is the physician who has the medical **practice** and is offering her or his **services** to patients who have **admitted their own inadequacies** by soliciting the physician.

Situational dependency:

ask for or try to obtain (something) from someone

making or expressing a plea, especially to someone in power or authority

It is the patient who has assumed the role of supplicant by seeking out service, scheduling an appointment, often waiting past the scheduled time, answering the physician's questions, and allowing an examination to occur.

Szasz–Hollender Model:

- Developed by two M.D.s, Thomas Szasz and Marc Hollender (1956)...further includes three sub-models

The Activity–Passivity Model

- Physician represents medical expertise, controls the communication flow between the two parties...patient is regarded as lacking important information and relying on the knowledge and judgment of the physician (parent and infant)

The Guidance–Cooperation Model

- Compared to previous model, the patient has increased involvement (providing information and decision taking)...The physician is less autocratic, patient's agreement to decisions is desired...but the physician retains the dominant position (parent and adolescent)

taking no account of other people's wishes or opinions

The Mutual Participation Model

- This model elevates the patient to full participant...patient knows more about her or his own situation
- Three essential traits that must be present:
 - Both should have approximately equal power (similar to relationship between two adults)
 - there must be some feeling of mutual interdependence (i.e., a need for each other)
 - they must engage in interaction that will in some ways be satisfying to both parties
- As it requires more from the patients it may be less appropriate for children or those who are mentally deficient, poorly educated, or very immature